AI-Powered HMS Synthetic Clinical Dataset

Encounter Note

Patient Name	Ta Phuong Xuan	**MRN**	MRN-0094
DOB	1964-08-23	**Gender**	Female
Encounter Date	2023-12-17	**Department**	Pediatrics
Attending Provider	RN Jamie Owens	**Status**	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Ta Phuong Xuan is a 61-year-old patient in Pediatrics presenting for follow-up of cough, wheeze, or fever with stable oxygenation. PMH includes recurrent cough or wheeze, immunizations reviewed, family educated on warning signs. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include albuterol inhaler 2 puffs Q4H PRN; acetaminophen 15mg/kg Q6H PRN. Allergies: sulfonamide allergy - hives.

Objective

Vitals: BP 134/84 mmHg, HR 76 bpm, RR 20/min, Temp 36.7 C, SpO2 95% RA, weight 65 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-04	Creatinine	1.41	mg/dL	0.7-1.3	High
2026-05-04	eGFR	68.0	mL/min/1.73m2	>=60	Normal
2026-05-04	Glucose	123.0	mg/dL	70-99	High
2026-05-04	HbA1c	6.0	%	<5.7	High
2026-05-04	BNP	207.0	pg/mL	<100	High
2026-05-04	AST	36.0	U/L	10-40	Normal
2026-05-04	ALT	31.0	U/L	7-56	Normal
2026-05-04	Potassium	4.3	mmol/L	3.5-5.0	Normal
2026-05-04	Hemoglobin	13.6	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: asthma or viral respiratory illness with hydration monitoring. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Monitor RR, SpO2, oral intake, and fever curve. Teach inhaler spacer technique. Return if increased work of breathing or poor intake. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. asthma or viral respiratory illness with hydration monitoring. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Pediatrics.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: encounter_note.